

City General Hospital

Department of Cardiology — Emergency Cardiac Unit

Patient Name:	Suresh Mehta	MRN:	MGH-2024-07832
Age / Sex:	61 years / Male	Date:	05-Mar-2026
DOB:	14-Jun-1964	Physician:	Dr. Priya Nair, DM (Cardiology)

EMERGENCY ADMISSION REPORT — ACUTE CHEST PAIN

Chief Complaint: Severe crushing chest pain radiating to the left arm and jaw, onset 2 hours ago.

History of Present Illness: Mr. Suresh Mehta is a 61-year-old male smoker (40 pack-years) with known hypertension and hypercholesterolaemia who presented to the emergency department with sudden onset severe retrosternal chest pain (9/10) radiating to the left arm and jaw, associated with diaphoresis, nausea, and dyspnoea. Pain began at rest approximately 2 hours prior to arrival. He denies prior similar episodes. On examination he is pale, diaphoretic, and in obvious distress. No previous cardiac history. Currently on Amlodipine 5 mg and Atorvastatin 40 mg.

VITAL SIGNS ON ADMISSION

Blood Pressure:	88/60 mmHg (Hypotensive)	Heart Rate:	112 bpm (Tachycardia)
Temperature:	37.0 °C	SpO2:	92% on room air
Respiratory Rate:	24 breaths/min	GCS:	15/15

ECG FINDINGS

12-lead ECG performed at 08:14 AM: **ST-segment elevation of 4–5 mm in leads II, III, and aVF** consistent with inferior wall ST-elevation myocardial infarction (STEMI). Reciprocal ST depression in leads I and aVL. Sinus tachycardia at 112 bpm. No left bundle branch block. QTc interval: 450 ms.

CARDIAC BIOMARKERS

Biomarker	Result	Normal Range	Unit	Flag
Troponin-I (hs-cTnI)	8.42	< 0.04	ng/mL	CRITICAL ↑↑
CK-MB	112	0 – 25	U/L	HIGH ↑
BNP	480	< 100	pg/mL	HIGH ↑
Myoglobin	420	< 90	ng/mL	HIGH ↑
D-Dimer	0.48	< 0.50	µg/mL	Normal
Serum Potassium	3.4	3.5 – 5.0	mEq/L	LOW ↓
Serum Sodium	138	135 – 145	mEq/L	Normal
Creatinine	1.3	0.7 – 1.2	mg/dL	HIGH ↑
PT/INR	1.1	0.9 – 1.1		Normal

CLINICAL NOTES

Highly elevated hs-cTnI (8.42 ng/mL) and CK-MB (112 U/L) with ST-elevation on ECG confirm an acute inferior STEMI. Haemodynamic instability (BP 88/60) suggests cardiogenic shock. Patient transferred immediately to catheterisation laboratory for emergency primary PCI (percutaneous coronary intervention). Dual antiplatelet

therapy (Aspirin 300 mg + Ticagrelor 180 mg) and unfractionated heparin initiated. Cardiothoracic surgery on standby.

PRELIMINARY DIAGNOSIS

Acute Inferior STEMI with Cardiogenic Shock. Risk: CRITICAL — Immediate PCI required.